

CASE MANAGEMENT TRAINEE HANDOUT

Advanced Concepts & Troubleshooting (Expanded)

Welcome to the advanced case management module. This handout covers critical nuances, operational bottlenecks, strategic elements, and new crucial topics that fall outside our standard training module. Mastery of these components ensures maximum case value and mitigates malpractice risks.

1. Operational Bottlenecks & Challenges

- **Medical Records & Billing Delays:** Providers using third-party fulfillment (e.g., ChartSwap, Ciox) often take 30-45 days to respond. Proactive follow-ups, utilizing statutory request timelines, and escalating to supervisors are necessary to prevent case stalling.
- **Client Disengagement:** Clients who miss appointments or fail to respond to updates damage their case value. Establish alternative contact methods (emergency contacts, email, text) early on, and emphasize how 'gaps in treatment' provide ammunition for defense adjusters.
- **Adjuster Stalling Tactics:** Insurance adjusters may frequently reassign cases or claim missing documentation. Keep meticulous logs of all transmissions (fax confirmations, email receipts) to combat intentional delays.

2. How Property Damage (PD) Affects Injury Claims

- **The MIST Paradigm (Minor Impact Soft Tissue):** Adjusters flag cases with minimal visible vehicle damage. They will use biomechanical arguments to assert that the impact force was insufficient to cause the claimed injuries.
- **Hidden Damage is Key:** A \$500 bumper scratch might conceal \$3,000 in unibody damage. Always request supplemental estimates and tear-down photos from the body shop. High PD validates the mechanism of injury.
- **Jury Perception:** Juries inherently link vehicle destruction to bodily harm. If PD is low, case managers must over-index on objective medical findings (MRIs, specialist reports) to offset the visual bias.

3. Navigating Liability Disputes

- **Word vs. Word Accidents:** Without independent evidence, carriers typically deny claims. Immediate action is required: canvass for nearby surveillance cameras, secure dashcam footage, and request 911 CAD (Computer-Aided Dispatch) reports for spontaneous witness utterances.

- **Comparative & Contributory Negligence:** Adjusters often try to place a percentage of blame on our client (e.g., 'failure to take evasive action'). Challenge these assertions immediately by utilizing point-of-impact analysis and referencing specific traffic statutes.
- **Police Report Discrepancies:** Police reports are not infallible. If an officer gets the narrative wrong, we must secure witness affidavits and scene photos to amend or counter the official report before submitting the demand.

4. Statute of Limitations (SOL) & Statutory Deadlines

- **The Ultimate Fatal Error:** Missing the Statute of Limitations extinguishes the client's right to recovery and invites a malpractice suit against the firm. The SOL must be verified, calendared, and double-checked within 24 hours of intake.
- **Notice of Claim (Government Entities):** If the at-fault party is a state, county, or municipal employee (e.g., city bus, police cruiser), standard SOLs do not apply. Strict Tort Claims Act notices are often required within 90 days to 6 months of the incident.
- **Uninsured/Underinsured Motorist (UM/UIM) Deadlines:** First-party claims against our own client's policy may have contractual deadlines that differ from state statutory deadlines. Always review the policy jacket immediately.

5. The Defense IME (Independent Medical Exam)

- **Not Actually 'Independent':** Insurance companies hire these doctors to minimize or deny the client's injuries. Trainees must understand this adversarial context and manage client expectations.
- **Client Preparation:** Clients must be prepped on what to say, what not to say, and how to conduct themselves. They should never guess on medical history and must be aware they are being observed from the moment they enter the parking lot.
- **Shadowing & Recording:** Whenever legally permissible, an IME should be recorded, or a firm representative/nurse should shadow the exam to ensure the defense doctor does not step out of bounds or misrepresent the duration of the exam.

6. File Documentation & Hygiene

- **The Golden Rule of Case Management:** 'If it's not in the file notes, it didn't happen.' Every phone call, text, or email with the client, adjuster, or medical provider must be logged immediately with a timestamp and summary.
- **Medical Chronologies:** Don't wait until the demand stage to build the timeline. Treat medical chronologies as living documents. Update them bi-weekly so the case is always ready for attorney review.

- **Proactive CYA (Cover Your Assets):** If a client insists on a course of action against our advice (e.g., stopping treatment early), document the conversation thoroughly and send a confirming email/letter to the client to protect the firm.

7. Additional Crucial Concepts

- **Managing the Eggshell Plaintiff:** Pre-existing conditions do not ruin a case; they complicate it. We must document the aggravation or exacerbation of the prior injury. Never allow clients to hide prior accidents from their treating physicians.
- **Subrogation and Liens:** Securing a \$100k settlement means nothing if a \$95k ERISA lien is attached. Begin identifying Medicare, Medicaid, and private health insurance liens on Day 1 to allow ample time for reduction negotiations.
- **Client Expectation Management:** Clients often Google 'settlement calculators' and develop unrealistic expectations. Set realistic baselines early, explaining how policy limits, liability percentage, and medical bills dictate final compensation.

8. LOP Inflation Monitoring

Letters of Protection (LOPs) carry inherent risks of bill inflation. Monitor provider billing against Usual and Customary Rates (UCR) to protect the client's net recovery. Use the tracking format below:

Provider Name	Procedure / Code	Billed Amount	UCR Est.	Variance / Notes
Dr. Smith (Ortho)	Spinal Injection	\$4,500	\$2,100	+\$2,400 (Flag for Reduction)
City Imaging	C-Spine MRI	\$2,200	\$900	+\$1,300 (High Priority)
Elite PT Rehab	Physical Therapy (x12)	\$3,600	\$2,400	+\$1,200 (Review notes)
Dr. Allen (Neuro)	Initial Consult	\$850	\$600	+\$250 (Standard)
General Hospital	ER Visit	\$6,800	\$4,500	Hospital Lien - statutory limits apply